

The Impact of Foreign Direct Investment Inflows on Life Expectancy

Jeffrey Wang

This paper estimates the effect of foreign direct investment on life expectancy using a two-way fixed effects panel model on World Bank data spanning 178 countries from 2001 to 2023. Controlling for GDP, GDP per capita, access to electricity, infant mortality, and education expenditure, the baseline FDI coefficient is statistically insignificant, indicating that FDI has no uniform effect on population health. The central finding is that the health return to FDI is moderated by host country income: the interaction between log FDI and GDP per capita is positive and significant at the 5 percent level with a coefficient of 0.00358, meaning a one percent increase in FDI raises life expectancy more in wealthier countries than in poorer ones. These results suggest that lower-income governments should not expect FDI inflows alone to improve population health, and that the sector composition and regulatory environment of incoming investment may matter more than its volume.

I. Introduction

Foreign direct investment (FDI) has become one of the defining economic forces in the modern era of globalization. Global FDI flows reached \$1.3 trillion in 2023, making it a central tool through which capital, technology, and employment cross borders (UNCTAD, 2024). Policymakers in developing nations routinely court foreign investors on the promise that inflows will raise incomes, build infrastructure, and improve the well-being of their populations. But whether FDI actually delivers on that expectation and the mechanisms in which investments interact with a country and its people is an important aspect of FDI research.

FDI has also become an essential instrument used in domestic and international politics. For many global leaders, attracting high-profile foreign projects serves as a signal to both voters and global markets, symbolizing stability and modernization. A prominent example is the expansion of the Taiwan Semiconductor Manufacturing Company TSMC, the chip hardware company at the heart of the AI boom, in the United States. In 2024 and 2025, TSMC dramatically increased its commitment to its Arizona facilities, with total planned investments reaching \$165 billion (TSMC, 2025). This project, the largest single foreign direct investment in U.S. history, has been leveraged by both the Biden and Trump administrations to signal a commitment to "national security" and "technological sovereignty" (U.S. Department of Commerce, 2024). By onshoring the production of advanced chips critical for AI and defense, the U.S. government uses this FDI as a part of its geopolitical and domestic economic narrative of success and innovation. The increasing political importance of FDI leads us to wonder, does FDI actually bring benefits to the livelihoods of people like we expect it to?

This paper studies one specific dimension: does foreign direct investment improve population health, measured by life expectancy? FDI provides not only employment and financial capital, but also a means of transferring technology and skills, and increasing access to global markets. These gains in productivity and income could, in turn, translate into better nutrition, greater access to healthcare, and longer lives. At the same time, FDI does not always take the form of hospitals or schools. Industries such as mining, logging, and oil drilling associated with FDI may prioritize profit over sustainable practices, resulting in significant environmental damage, and a number of specific medical conditions have been documented as resulting from the activities of extractive industries, including occupational and environmental hazards such as exposure to pollution, pulmonary diseases due to dust inhalation, and respiratory diseases and cancer linked to gas flaring (Calain, 2012).

This paper examines the FDI–life expectancy relationship globally, using World Bank panel data spanning 2001 to 2023. It employs a time-entity fixed effects model to control for unobserved country-level and year-level characteristics that might confound the relationship. It also models the interaction between FDI and GDP per capita, allowing the estimated health effect to vary by income level rather than assuming a uniform global effect, allowing us to see if income changes how countries are affected by FDI and to quantify any effect.

In the research, I find that FDI does have a statistically significant effect on life expectancy, but that their relationship depends on the interaction between FDI and GDP per capita. A one thousand dollar increase in GDP per capita increases the effect that a 1% increase

in FDI has on life expectancy by 0.00358 years, holding constant time and country invariant factors, GDP, access to electricity, infant mortality, health expenditure, and education.

The remainder of this paper is organized as follows. Section II reviews the existing literature on foreign direct investment and population health outcomes, situating this paper within the broader debate on how capital flows shape well-being across income levels. Section III describes the World Bank panel dataset, covering the construction of key variables, the time period under study, and relevant summary statistics. Section IV presents the econometric model, detailing the time-entity fixed effects specification, the log transformation of FDI, and the interaction term with GDP per capita used to capture heterogeneity across country income levels. Section V reports the results, including the estimated marginal effects of FDI on life expectancy and the robustness of findings across alternative specifications. Section VI summarizes the findings, discusses their policy implications, and acknowledges the limitations of the analysis, including the endogeneity concerns that accompany cross-country panel estimation.

II. Literature

Previous literature studying the relationship between foreign direct investment and population health has grown considerably over the past two decades, though it remains fragmented across countries, methodological approaches, and income groups. This allows our paper to organize around the foundational work on health as a determinant of FDI, early panel evidence on FDI's effect on health, and more recent research that reveals how the relationship varies by income level and sector of investment.

The early quantitative work on this relationship studied the question in reverse, not whether FDI improves health, but whether healthier populations attract more foreign investment. Alsan, Bloom, and Canning (2004) analyzed World Bank panel data from 74 developing countries from 1980 to 2000 and found that gross FDI inflows have a strong and positive effect on population health in low and middle income countries, as a 1 year increase in life expectancy is associated with a 9% rise in gross FDI inflows. Their research interpretation is based on human capital theory: healthier workers are more productive and accumulate more job experience, making them attractive to foreign investors. Their paper focused on the reverse side of the FDI–health relationship, but their findings raise an important methodological challenge for our research. Specifically, FDI may be endogenous and any observed correlation between FDI and life expectancy may partly reflect healthier countries attracting more investment rather than investment causing better health outcomes. This endogeneity concern motivates the instrumental variable and fixed-effects approaches adopted in subsequent literature and in this paper. While Alsan et al. used a standard OLS panel regression, our paper addresses this reverse causality through a time-entity fixed effects model, which absorbs country-level characteristics that simultaneously attract FDI and influence life expectancy.

Burns, Jones, Goryakin, and Suhrcke (2017) made the most important methodological contribution in this literature. Using World Bank panel data from 85 low and middle income countries between 1974 and 2012, looking into life expectancy and mortality rates as health outcomes and net FDI inflows alongside GDP per capita, trade openness, and health expenditure as key regressors, they employed an instrumental variable approach using origin-country determinants of FDI as instruments. Their main finding is that FDI has a beneficial effect on life expectancy in low and middle-income countries and that controlling for endogeneity is extremely

important. They also find preliminary evidence that secondary sector FDI such as manufacturing is harmful to health, while primary and tertiary sector FDI are not. Our paper is similar to Burns et al. in its focus on identifying the direction of causality and in using life expectancy as the outcome variable. Our papers differ in scope, as our sample includes both high and low-income countries.

More recent literature has expanded both the geographic and methodological scope of this inquiry. Examining Turkey during its period of economic openness, with an autoregressive distributed lag model and World Bank data, there was a positive relationship between FDI and life expectancy in the Turkish context, and a similar positive effect for trade openness (Bese 2021). While this finding is consistent with the broader pattern for middle-income countries, the single-country focus limits its generalizability. A global analysis is important to understand whether Turkey's experience is representative or exclusive.

Together, the literature leaves us with some tentative conclusions: FDI tends to benefit health in lower-income countries and harm or leave unaffected health in higher-income ones; the type of FDI matters, with extractive and manufacturing investment posing greater health risks than service sector investment; and models that fail to account for endogeneity underestimate the true effect. What remains less explored is a comprehensive global analysis spanning the full breadth of available country-year data, using a fixed-effects framework to control for unobserved heterogeneity, and explicitly modeling how the income level of the host country modifies the estimated health effect of FDI, which this paper aims to address.

III. Data

The data for our research uses country-year panel data from the World Bank's World Development Indicators (WDI) Database, a cross-country panel dataset compiled primarily from the International Monetary Fund's Balance of Payments statistics, supplemented by data from the UN Conference on Trade and Development (UNCTAD) and official national statistical sources (World Bank, 2025). The WDI is widely used and trusted as it standardizes variable definitions and collection methodologies across countries, making cross-country comparisons more reliable than assembling country-specific sources independently. The database is also used by a majority of the literature around FDI, including all the papers in the literature review. We classify the primary dependent variable life expectancy as how many years a person is expected to live given the mortality patterns in their country. Our main independent variable foreign direct investment is defined as net FDI inflows, measured in current U.S. dollars as reported in the balance of payments. Our analysis compares net FDI inflows against other measures of FDI such as total net FDI and FDI as a percentage of GDP, and we transform FDI using a logarithmic function in order to normalize the scale of the variable. This also allows our interpretation of the regression result meaningful, as we can compare a one percent increase in FDI rather than a 1 dollar change.

The original panel data spans 217 countries from 1960 to 2025, with 14322 rows of observations. When we limit the data to only the observations which contain values for our variables we end up with 3930 observations from 178 countries across 2001 to 2023, due to reporting not being consistent across years as well as inherent differences in cross-country data availability. Additionally, in the regressions and models where our FDI variable is logged, only observations with positive FDI values are included due to non-positive values being undefined in the natural logarithmic function. As a result, our analysis using the logged FDI variable includes

3671 observations across the same timeframe and number of countries. Our approach to restricting our dataset to the appropriate observations was through defining new variables which differentiate between whether an observation meets the requirements or not, combined with condition statements which limit the data used in the regression based on the value of those variables.

Our regression analysis controls for several country-level characteristics that prior literature identifies as confounders of the FDI–life expectancy relationship. GDP controls for the overall size of the economy. GDP per capita captures the average material standard of living, which is closely linked to both health outcomes and FDI attractiveness, and also enters the model as the moderating variable in the interaction term with FDI. Access to electricity proxies for basic infrastructure development and energy access, which has documented effects on health outcomes including indoor air quality and the ability to refrigerate medicine. Infant mortality is included as a broader indicator of existing healthcare system quality and child health conditions. Education expenditure as a share of government spending captures investment in human capital, which affects both health literacy and long-run economic development. Summary statistics for all variables are reported in Table 1.

Looking at Table 2 and 3, which include summary statistics of the variables used in this paper, we find a large disparity in life expectancy itself. High income countries have a mean life expectancy of 79.47 years with a relatively tight standard deviation of 2.95, reflecting a convergence among wealthy nations toward a health frontier. Low income countries average just 60.07 years, a difference of nearly two decades, and substantially more variation across countries ($SD = 6.33$) and a minimum of 14.66 years, which likely reflects conflict or famine conditions in the worst-affected observations. This 19-year gap in average life expectancy is the outcome this paper seeks to explain, and the raw descriptive statistics suggest that income level alone is a powerful predictor of where a country sits in that distribution.

FDI flows differ enormously in both scale and relative importance as. High income countries receive an average of \$33.01 billion in FDI inflows, compared to just \$670 million in low income countries. However, when measured as a share of GDP, the picture partially reverses: FDI as a percentage of GDP averages 12.63% in high income countries but only 3.69% in low income ones, though with dramatically higher variance in the high income group ($SD = 43.44$ versus 7.32). This high variance in FDI as a share of GDP among high income countries is likely driven by small, wealthy financial hub economies such as Luxembourg, Singapore, and Ireland, where FDI inflows are large relative to domestic output. The minimum of zero for FDI/GDP in both groups reflects years where net FDI inflows were negligible or negative, consistent with the disinvestment episodes discussed in the data section.

IV. Model

The primary challenge in estimating the effect of FDI across 178 countries and over 20 years is dealing with the bias that comes from each observation. Every country is unique, which adds bias from omitted variables we cannot account for like the culture or the behavior of citizens, and global fluctuations that affect all countries like the 2008 financial crisis can also bring additional bias. In order to resolve these issues, we take advantage of our panel data and employ a two-way fixed effects model in order to resolve time and country characteristics. Our Model:

$$lifeExpectancy_{it} = \beta_0 + \beta_1 logFDI_{it} + \beta_2 GDPpc_{it} + \beta_3 GDPBil_{it} + \beta_4 Electricity_{it} + \beta_5 InfantMortality_{it} + \beta_6 healthExpenditure_{it} + \beta_7 Education_{it} + \beta_8 logFDIb_{it} * GDPpc_{it} + a_i + \delta_t + e_{it}$$

represents countries with the subscript i and time with the subscript t , meaning variables like life expectancy and FDI, and the error term vary across time and country, while a variable that represents country fixed-effects will only vary across countries. This allows us to capture time-invariant factors within a_i , meaning country specific characteristics that do not change significantly within a country over the timeframe of our research will be accounted for. Without this, we might be measuring changes in life expectancy due to differences between countries like how often war or natural disasters affect a country instead of how different inflows of FDI affect life expectancy within that country. We also capture year fixed effects using δ_t which absorbs global changes that affect all countries simultaneously in a given year. This means controlling for global shocks like the COVID-19 which had a direct negative effect on life expectancy in 2020 and 2021, but also global financial crises or medical technologies that may raise life expectancy globally in a given period. Without year fixed effects, these common trends would be absorbed into the error term and potentially correlated with FDI flows, which also respond to global events, producing biased estimates.

When employing a fixed effects model, we must validate whether capturing these fixed effects is appropriate. If a country's characteristics are not correlated with any of our other variables, then removing it using fixed effects would make the estimators for our variables inefficient. To measure this, we must run the Hausman specification test which compares our fixed effects model to a random effects model. The null hypothesis of the Hausman test is that the random effects model is efficient and that our unobserved country effects a_i is not correlated with our independent variables. If the Hausman test reports a low p-value, then we reject the null as there is correlation and employing the fixed effects model provides a more efficient estimate. We also include an interaction term between the logged FDI and GDP per capita in order to measure the effect of FDI and how it might change across countries of different income levels. If the estimator of the interaction term is significant, then we can say that low and high income countries are affected differently from FDI, and that a positive and significant estimate means that high income countries benefit more from FDI compared to low income countries, and vice versa.

To allow us to compare our main model we incorporate multiple types of regressions in Table 4. We show an ordinary least square regression as the first layer, adding on control variables to account for omitted variable bias and our interaction term to measure the change in the FDI-life expectancy relation when income levels vary. We also show the fixed effects model with just the country fixed effects as well as with the time fixed effects, and allowing us to view the impact of bringing on additional time fixed effects.

Another concern is reverse causality, as healthier populations attract more FDI (Alsan, 2004). This means FDI may be endogenous in our model, and even the fixed effects estimators do not resolve this bias. We test an instrumental variable approach to deal with endogeneity using the amount of trade a country does. Countries that are more open to trade offer foreign investors access to global supply chains, lower tariff barriers on imported inputs, and easier repatriation of profits, all of which incentive foreign investors to spend and invest in those markets. Trade and FDI inflows move together because both of them are reflective of a country's integration and openness to participating in the global economy. The same policies that reduce

barriers to trade, such as joining the World Trade Organization or signing bilateral trade agreements, also reduce barriers to allow capital flow from foreign investors. Even in empirical literature, we find that trade openness and FDI are positively and significantly correlated in countries like Turkey (Bese, 2021). While some would argue that trade has some correlation with life expectancy through trade of medical supplies and equipment, these channels are substantially absorbed by the control variables already included in the model. GDP and GDP per capita control for the income-raising effects of trade. Health expenditure controls for the availability of imported medical goods and services. Infant mortality controls for the baseline health environment. We acknowledge this assumption is not directly testable, but it is supported by the logic that the income and health-system channels of trade are appropriately proxied by the included controls. This logically explains the relevance of trade as an instrumental variable, and that the effect of trade excludes effects to life expectancy.

A valid instrument needs to be strongly correlated with the independent variable which we suspect to be endogenous (FDI), and not correlated with our dependent variable, life expectancy, other than through trade's correlation with FDI. We first would need to test the significance of the correlation between Trade and FDI using an F-test, where the typical threshold of $F > 10$ (Staiger and Stock, 1997) marks the instrument as being strong, but F-test values above 3.2 are also relevant. Another instrument, foreign aid, was tested to be the second instrumental variable for FDI, but aid was not sufficiently correlated to aid as it brought the joint-F test of trade and aid to be lower, and when regressed on FDI it was not significant. Due to this, we remove the use of aid as an instrumental variable, meaning that we have just one instrument variable, and that our model would be just-identified with just the trade variable as an instrument for FDI.

Finally, we need to test for endogeneity by getting the predicted for FDI using the instrument variable. We run a regression with FDI as the main dependent variable, along with the instrumental variable, trade, and other exogenous variables in the model. Then, we take the predicted residual values from this regression and add them as a variable in our main model. If the predicted values are significant, then we know that FDI is in fact endogenous in our model and that we need to include instrumental variables in our regression in order to have non-biased estimators.

As a robustness check on the relationship between FDI and life expectancy, we also estimated specifications using lagged values of log FDI, specifically lags of one, two, and three years to test whether the health effects of foreign investment materialize with a delay rather than within the same year. The motivation for this check is conceptually reasonable: FDI may take time to translate into employment, infrastructure, and ultimately health outcomes, meaning a contemporaneous model could understate or misattribute the true effect. One important thing to note is that when lagging data, we will lose some observations from the early years as there is no previous data to compare the lags to.

Other concerns for our data is heteroskedasticity and serial correlation, both of which are expected in our panel data. Between our countries, it is reasonable to assume that all 178 countries we study will not have the same variance for their error term because the countries differ enormously in size, income, and development. For serial correlation, observations in a panel dataset from the same country across different years are not independent of each other, so life expectancy in the United States in 2001 will be closely related to its life expectancy in 2002. Both of these make the standard errors that we report unreliable, and the way we resolve these

issues is using clustered standard errors, which will provide us robustness for our panel data standard errors.

V. Results

The regression results reported in Table 4 tells us about how the estimated effect of FDI on life expectancy changes as the model accounts for more sources of bias. In Model 1, the simple OLS regression without controls, the coefficient on log FDI is 1.782 and is significant at the one percent level, suggesting that a one percent increase in FDI is associated with roughly 1.782 additional years of life expectancy. This estimate is almost certainly biased: wealthier, healthier countries attract more foreign investment, so the correlation between FDI and life expectancy captures cross-country differences in development rather than any within-country effect of FDI itself. Once control variables are added in Model 2, the coefficient falls sharply to 0.309, which is still significant at the one percent level but much smaller than the first model, confirming that a substantial portion of the Model 1 estimate reflected omitted variable bias. When country fixed effects are introduced in Model 3, the coefficient drops further to 0.037 and loses statistical significance. This progression shows that much of the observed FDI–life expectancy correlation in a simple regression model is driven by stable differences between countries rather than by changes in FDI within a given country over time.

The two-way fixed effects specification in Model 4 adds year fixed effects to absorb global shocks, and does not have the interaction term in order for us to compare it against our main model (5). The log FDI coefficient is -0.025 and remains statistically insignificant in model 4, reinforcing that FDI does not have a uniform, unconditional effect on life expectancy once country and year are controlled for.

In Model 5, our main model, we add an interaction term between log FDI and GDP per capita. The interaction coefficient is 0.00358 and is significant at the 5 percent level, indicating that the health return to FDI rises with host country income level. For example, at a GDP per capita of \$10,000, the estimated marginal effect of a one percent increase in FDI on life expectancy is approximately -0.039 years, which is statistically indistinguishable from zero. At a GDP per capita of \$50,000, the marginal effect becomes 0.104 years, a small but positive and meaningful gain. The adjusted R-squared of Model 5 is 0.813, meaning the model accounts for a large 81 percent of the variance in life expectancy across the sample..

Among the control variables in Model 5, infant mortality is the most significant variable at the 1 percent level, consistent with its role as a broad indicator of baseline health system quality and child health conditions. Education expenditure and GDP are both negative and significant at the 5 percent level which may reflect the fact that larger economies and more educated economies are not necessarily healthier once income per capita and other controls are held constant. Electricity access and health expenditure are not statistically significant in the preferred specification, though electricity access was significant in earlier models, suggesting its effect is partially absorbed by the fixed effects or correlated controls.

The Hausman test reported in Table 5 supports the approach of using time and country fixed effects, producing a chi-squared statistic of 261.02 ($p = 0.0000$), which strongly rejects the random effects specification and confirms that country-specific unobserved characteristics are correlated with the regressors. The fixed effects estimator is therefore the appropriate choice. The robustness checks reported in Table 4b and Table 5 support the findings that both an instrumental variable approach and a lagged effects approach are not appropriate for the model . The

instrumental variable model in Table 4b uses trade openness as an instrument for log FDI. The first-stage F-statistic is 7.99, which falls below the conventional Staiger-Stock threshold of 10, indicating a weak instrument concern. Another important result to mention is that when running the relevance test with both trade as well as another instrument, foreign aid, we found that they were perfectly multicollinear, and so we dropped aid as an instrument. The endogeneity test reported in Table 5 produces a p-value of 0.621, meaning we fail to reject the null that FDI is exogenous in the fixed effects model.

This result validates the use of the fixed effects estimates as the paper's primary specification rather than the IV estimates, which carry weak-instrument bias. The lagged effects model in Table 4b, Column 2 includes log FDI at lags of one, two, and three years, but none of the lagged coefficients are statistically significant, and the joint F-test for the lagged terms produces a p-value of 0.371, reported in Table 5. This result suggests that the health effects of FDI do not materialize with a meaningful delay, and that the income-heterogeneity channel identified in the preferred model is not obscured by timing issues in the data.

VI. Conclusion

This paper examines the relationship between foreign direct investment and life expectancy using a two-way fixed effects panel model estimated on World Bank data spanning 178 countries from 2001 to 2023. We find that FDI does not have a uniform effect on life expectancy. The key result is the interaction between log FDI and GDP per capita, which is positive and statistically significant at the 5 percent level ($\beta = 0.00358$) in the main model. This means that the health gains from FDI are more effective in higher-income countries. A one percent increase in FDI is expected to provide a larger gain in life expectancy in wealthier countries than in poorer ones. The Hausman test rejects the random effects model for the fixed effects model, showing that country-specific unobserved characteristics are important and correlated with the regressors, making the fixed effects the appropriate model.

The policy implication is that lower-income governments should not expect FDI inflows to automatically improve population health. At low GDP per capita, the marginal effect of FDI on life expectancy is essentially zero, suggesting that the mechanisms through which FDI improves health, like higher wages, infrastructure, technology transfer, either fail to materialize or are offset by environmental and labor harms. Relating these findings of Burns et al. (2017), who identify manufacturing FDI as harmful to health while services FDI is neutral or positive, we suggest that lower-income governments would be better served by policies that screen for the sector and labor standards of incoming investment rather than maximizing aggregate inflows. Higher-income countries do see measurable health benefits from FDI, though diminishing returns to economic inputs at high baseline life expectancy levels mean the absolute gains are modest.

There are a few limitations for the interpretation of these results. First, the instrumental variable strategy intended to address the endogeneity of FDI was not successful. Trade openness, measured as total trade as a percentage of GDP, was attempted as an instrument for log FDI on the grounds that trade liberalization increases FDI attractiveness without directly affecting life expectancy through channels not already controlled for. However, the first-stage F-statistic on trade openness is 7.99, which falls below the Staiger-Stock (1997) threshold of 10 conventionally used to ensure strong instruments. So when the follow-up endogeneity test found that FDI is not endogenous and ruled out an instrumental variable, it is possible that this result may be affected by using the wrong instrument. The other instrument we tested, foreign aid, was also found to be

perfectly collinear with trade, causing it to be dropped from significance tests, which could have been prevented if better instrumental variables were found.

Second, the exclusion of observations may introduce sample selection bias. With non-positive FDI values being dropped when applying the log transformation, we lose information about what happens to countries in those observations. Negative net FDI inflows occur when disinvestment, loan repayments by foreign affiliates, or capital repatriation exceeds new inflows in a given year (World Bank, 2025), and negative inflows are not random. They are more common during periods of crisis, which is associated with declining health outcomes. Dropping these observations therefore selectively removes country-years that may be most informative about the negative risks of FDI, potentially biasing the estimated effect of FDI on life expectancy upward. The unbalanced nature of the panel also means that results may be partially driven by the subset of countries with more complete data reporting rather than by the full global sample. This is most apparent in the electricity access variable, which has 6,532 non-missing observations compared to 13,854 for life expectancy, which reflects the weaker statistical capacity of lower-income countries, which are less likely to consistently report certain indicators. Since lower-income countries are disproportionately dropped from the analytical sample due to missing control variables, the effective sample skews toward higher-income nations with more complete records. This means that our results may overrepresent the experience of countries for which FDI is already found to be more beneficial, and underrepresent the lower-income countries for which the policy stakes are highest.

For future research, an extension of this paper disaggregating FDI by sector; separating manufacturing, extractive, and service industries would help us test whether the null result for lower-income countries reflects harmful manufacturing and extractive investment offsetting beneficial services investment, as Burns (2017) suggests, or if FDI simply has low returns to life expectancy in low income countries.

VII. Appendix

Table 1: Variable Description

<i>Life Expectancy</i>	Life Expectancy At Birth (years)
<i>logFDI</i>	Foreign Direct Investment Inflows (log USD)
<i>FDIpGDP</i>	Foreign Direct Investment Inflows (% of GDP)
<i>FDIBil</i>	Foreign Direct Investment Inflows (Billions USD)
<i>GDPpcK</i>	GDP Per Capita (thousands USD)
<i>GDPt</i>	GDP (trillions USD)
<i>Electricity</i>	Access to Electricity (% of Population)
<i>Infant Mortality</i>	Mortality rate, infant (per 1,000 live births)
<i>Health Expenditure</i>	Health Expenditure (years)
<i>Education</i>	Compulsory Education (years)
<i>Trade</i>	Trade (% of GDP)

Table 2: Summary Statistics

Summary statistics					
	N	Mean	SD	Min	Max
LifeExpectancy	3930	70.84	8.35	14.66	85.71
FDIpGDP	3930	5.71	23.53	-391.56	452.22
FDIBil	3930	10.06	40.75	-343.4	733.83
GDPpc	3930	13351.29	18141.32	243.08	112417.88
GDPb	3930	410.93	1660.41	.03	21955.25
Electricity	3930	82.87	27.27	1.3	100
Infant	3930	24.56	23.42	1.3	239.9
Health Expenditure	3930	6.27	2.78	1.22	27.09
Education Minimum	3930	9.47	2.28	4	17

Table 2b: Additional restrictions to observations with Trade

Summary statistics

	N	Mean	SD	Min	Max
LifeExpectancy	2286	67.91	7.53	14.66	81.04
logFDI	2286	20.18	2.21	11.45	26.56
FDIpGDP	2286	4.09	4.57	0	56.29
FDIBil	2286	4.85	21.04	0	344.07
GDPpcK	2286	4.18	3.71	.32	23.69
GDPt	2286	.22	1.09	0	17.61
Electricity	2286	76.55	29.74	1.3	100
Infant	2286	31.66	23.25	1.9	239.9
HealthExpenditure	2286	5.67	2.42	1.41	22.3
EducationMinimum	2286	9.3	2.38	4	17
Trade	2286	76.33	37.66	2.47	347.72

Table 3A: Descriptive Statistics - High Income Countries

	N	Mean	SD	Min	Max
LifeExpectancy	904	79.47	2.95	69.05	85.71
logFDI	904	22.58	2.18	14.51	27.32
FDIpGDP	904	12.63	43.44	0	452.22
FDIBil	904	33.01	68.43	0	733.83
GDPpcK	904	37.7	18.15	16.15	110.87
GDPt	904	1.11	2.87	0	21.96
Electricity	904	99.96	0.36	92.2	100
Infant	904	5.27	3.72	1.3	24.1
HealthExpenditure	904	7.73	2.83	1.6	18.52
EducationMinimum	904	10.27	1.67	6	15

Table 3B: Descriptive Statistics - Low Income Countries

	N	Mean	SD	Min	Max
LifeExpectancy	721	60.07	6.33	14.66	73.14
logFDI	721	18.89	1.86	11.45	24.32
FDIpGDP	721	3.69	7.32	0	103.34
FDIBil	721	0.67	2.42	0	36.5
GDPpcK	721	0.85	0.31	0.24	1.36
GDPt	721	0.03	0.12	0	1.7
Electricity	721	40.06	27.46	1.3	100
Infant	721	57.41	21.88	14	239.9
HealthExpenditure	721	5.15	2.34	1.47	21.92
EducationMinimum	721	7.75	1.89	5	17

Table 4: Main Regression Results

Table 4: Impact on Life Expectancy					
VARIABLES	(1) OLS	(2) OLS Controls	(3) Country FE	(4) 2-Way FE	(5) Main Model
logFDI	1.782*** (0.175)	0.309*** (0.104)	0.0371 (0.0607)	-0.0254 (0.0356)	-0.0753 (0.0493)
GDP (trillions \$USD)		-0.0716 (0.0991)	-0.0788 (0.0618)	-0.145** (0.0614)	-0.155** (0.0666)
GDP per capita (thousands \$USD)		0.122 (0.0874)	0.142* (0.0736)	0.0858*** (0.0253)	-0.00161 (0.0498)
Electricity		0.0529*** (0.0114)	0.0281* (0.0155)	0.00403 (0.0151)	0.00506 (0.0152)
Infant Mortality		-0.229*** (0.0178)	-0.214*** (0.0111)	-0.186*** (0.0207)	-0.187*** (0.0204)
Health expenditure		-0.00822 (0.0822)	0.0138 (0.0583)	-0.0853 (0.0541)	-0.0871 (0.0544)
Education		-0.0214 (0.0707)	0.0483 (0.0450)	-0.124*** (0.0475)	-0.122** (0.0475)
LogFDI X GDPpc		-0.00136 (0.00385)	0.00172 (0.00238)		0.00358** (0.00171)
Constant	33.90*** (3.822)	64.78*** (2.828)	70.17*** (1.646)	74.99*** (1.733)	76.03*** (1.853)
Observations	3,671	3,671	3,671	3,671	3,671
Adjusted R-squared	0.285	0.901	0.758	0.812	0.813
Number of Countries			178	178	178
Country FE			YES	YES	YES
Year FE				YES	YES

Robust standard errors in parentheses

*** p<0.01, ** p<0.05, * p<0.1

Table 4b: Additional Regression Tests

VARIABLES	(1) IV Model	(2) Lagged Effects
logFDI	-0.294 (0.297)	-0.0449 (0.0485)
Lag 1 logFDI		-0.0226 (0.0227)
Lag 2 logFDI		0.0131 (0.0231)
Lag 3 logFDI		-0.0247 (0.0284)
GDP (trillions \$USD)	-0.350** (0.146)	-0.153** (0.0623)
GDP per capita (thousands \$USD)	-1.667 (1.207)	0.0383 (0.0457)
Electricity	0.0331*** (0.00729)	0.00525 (0.0166)
Infant Mortality	-0.206*** (0.00479)	-0.195*** (0.0198)
Health Expenditure	-0.115*** (0.0224)	-0.0895 (0.0633)
Education	-0.103*** (0.0242)	-0.124** (0.0493)
LogFDI X GDPpck	0.0833 (0.0537)	0.00232 (0.00161)
Constant	78.83*** (5.578)	76.27*** (2.295)
Observations	2,286	3,289
Number of Countries	129	176
Country FE	YES	YES
Year FE	YES	YES
Adjusted R-squared	0.8757	0.820

Standard errors in parentheses

*** p<0.01, ** p<0.05, * p<0.1

Table 5: Additional Tests

Test Name	(1) Hausman	(2) IV Relevance	(3) IV Endogeneity	(4) Lags
P-value	0.0000	0.0055	0.6207	0.3711
F-stat		7.99	0.25	1.05
Chi-squared statistic	261.02			

VIII. References

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